Twin Pregnancy 07. Delivery Timing and Route

Companion reading handout for simultaneous listening.
Audio course on twin pregnancy. Episode seven. Delivery timing and route.

Source base

This episode is based on Gabbe chapter 32, the Israeli position paper on management of delivery in multiple gestation, the local twin delivery protocol, and the position papers on VBAC, breech delivery, operative vaginal delivery, and external cephalic version.

Opening frame

Let us begin with the central idea.

Timing of delivery in twins balances two curves. One curve is neonatal prematurity risk. The other is ongoing intrauterine risk. In twin pregnancy, the intrauterine risk curve rises earlier than it does in singleton pregnancy, especially when the placenta is monochorionic.

That is why uncomplicated twins are not managed toward 40 or 41 weeks.

For dichorionic diamniotic twins, the Israeli delivery position paper recommends delivery around 37 to 39 weeks. The local protocol is more operational: deliver at 38 weeks, do not exceed 40 weeks, and treat the pregnancy from 38 weeks with postterm-like attention.

For monochorionic diamniotic twins, delivery is earlier, around 36 to 37 weeks in the position paper, and 36 to 36 plus 6 in the local protocol. The reason is not that the fetuses are necessarily immature or sick at 36 weeks. The reason is that late stillbirth risk is higher in monochorionic twins, even when they look uncomplicated.

For monochorionic monoamniotic twins, delivery is earlier still, generally 32 to 34 weeks, with the local protocol recommending 32 to 33 weeks by cesarean after corticosteroids, and magnesium if delivery is before 32 weeks. This is the cord-risk pregnancy. The problem is not simply placental vascular sharing. It is shared amniotic space.

Now route of delivery

Now route of delivery.

The decision is not "twins equals cesarean" or "twins can deliver vaginally." The decision depends on presentation, gestational age, estimated weights, monitoring feasibility, operating room availability, anesthesia, and the team's skill with second-twin maneuvers.

Twin A is the gatekeeper.

If the first twin is non-cephalic at delivery, cesarean is recommended. The reason is mechanical and safety based. The first fetus must negotiate the cervix and pelvis first. A non-cephalic first twin creates a risk profile closer to planned breech birth, with added twin complexity.

If twin A is cephalic and twin B is cephalic, vaginal delivery is generally recommended if both fetuses can be monitored and there is no usual obstetric contraindication. This is the cleanest scenario.

If twin A is cephalic and twin B is non-cephalic, vaginal delivery can still be appropriate. This is the scenario where resident education matters most.

After twin A delivers, the uterus changes. Twin B can change lie. The membranes may rupture. The cord can prolapse. The placenta can abruption. The cervix can begin to contract down. Twin B is more vulnerable than twin A in this interval.

The Israeli position paper allows vaginal delivery of vertex-nonvertex twins when the team is experienced in delivering a non-cephalic second twin and twin B is not more than 20 percent larger than twin A. From 32 weeks, neonatal complication rates are similar between planned vaginal and planned cesarean delivery when twin A is cephalic, regardless of twin B presentation, in the cited framework.

The local protocol gives sharper exclusions for vertex-nonvertex vaginal delivery. Do not proceed if twin B is more than 20 percent larger than twin A, if twin B is below 1500 grams or above 3500 grams, or if gestational age is below 32 weeks.

Now second twin management

Now second twin management.

After twin A is delivered, immediately determine twin B presentation and heart rate. Ultrasound should be available. If twin B is cephalic and the tracing is reassuring, spontaneous delivery can be awaited, and oxytocin can be used when appropriate to shorten the interval.

If twin B is non-cephalic, breech extraction is often preferred over external cephalic version in experienced hands. Why? External version after delivery of twin A can cause fetal distress, cord prolapse, compound presentation, and failure. Breech extraction controls the fetus and uses the fact that the cervix is fully dilated.

The breech delivery position paper explicitly says that singleton planned-cesarean breech policy does not apply to second twin scenarios. That is because second twin breech is a different anatomy and timing problem than singleton term breech.

External cephalic version, or ECV, is generally contraindicated in multiple gestation as a background procedure, but the delivery paper allows consideration after twin A in selected second-twin circumstances. That apparent conflict is resolved by context. Antepartum ECV in an ongoing twin pregnancy is different from an intrapartum maneuver after the first twin has delivered.

Now prior cesarean

Now prior cesarean.

Twin pregnancy is not an absolute contraindication to trial of labor after cesarean when the previous uterine incision is a documented lower-segment transverse incision and twin A is cephalic. The VBAC position paper states there is no contraindication to TOLAC in twin pregnancy. The local protocol requires senior physician approval and informed consent.

Now operative vaginal delivery

Now operative vaginal delivery.

The operative vaginal delivery position paper and local vacuum protocol allow special judgment for the second twin, including circumstances where the usual station rule is modified. This is not permission for casual vacuum use. It reflects the fact that a second twin may need expedited delivery in a unique intrapartum setting, and that senior skill is required.

Pause and summarize.

Delivery timing follows chorionicity: DCDA later, MCDA earlier, MCMA earliest. Route follows twin A first, then twin B, estimated weights, gestational age, monitoring, and team skill. Twin A non-cephalic usually means cesarean. Twin A cephalic allows planned vaginal delivery in many cases, even if twin B is non-cephalic, when criteria and expertise are present.

Now final synthesis

Now final synthesis.

Key point

First, twins are delivered before singleton postterm logic because intrauterine risk rises earlier.

Second, chorionicity sets the timing framework.

Third, presentation of twin A is the main route-of-delivery gatekeeper.

Fourth, safe vaginal twin delivery requires second-twin skill, monitoring, anesthesia, operating-room readiness, and a team that can change course quickly.

End of episode seven.